

WEIGHT LOSS

Semaglutide Protocol

Candidacy, titration, administration, monitoring and the documentation that makes this a medical program

DOCUMENT WL-001	VERSION 1.0	EFFECTIVE January 2026	REVIEW CYCLE Annual
APPLIES TO Prescribing clinicians and clinical staff	OWNER Medical Director	CLASSIFICATION Confidential	SUPERSEDES All prior versions

1 Purpose & Scope

This protocol governs semaglutide therapy for weight management from initial candidacy through maintenance. It applies to FDA-approved branded product and, where the practice uses it, to compounded semaglutide — with the additional requirements in WL-006 applying in full to the latter.

ABSOLUTE CONTRAINDICATIONS

Personal or family history of medullary thyroid carcinoma, or Multiple Endocrine Neoplasia syndrome type 2. Known hypersensitivity to semaglutide or any excipient. Ask the question explicitly, record the question and the answer, and do not proceed on a shrug.

THIS IS A PRESCRIBING RELATIONSHIP

Semaglutide is a prescription medication. A questionnaire and a monthly vial is not a medical program and is precisely the model regulators are moving against. Every element below — evaluation, documented rationale, monitoring, follow-up — is what distinguishes the two.

2 Candidacy & Baseline Evaluation

Full candidacy criteria are in WL-003. Before a first prescription is written, all of the following must be complete and in the chart.

- ☐ Height, weight and BMI measured in the practice — not self-reported.
- ☐ Blood pressure and resting heart rate recorded.
- ☐ Complete medication list including insulin, sulfonylureas, oral contraceptives and thyroid medication.
- ☐ Personal and family history screen for medullary thyroid carcinoma and MEN2, documented.
- ☐ History of pancreatitis, gallbladder disease, gastroparesis or bariatric surgery.
- ☐ Diabetic retinopathy status in patients with diabetes.
- ☐ Pregnancy status, contraception, and pregnancy intent discussed and recorded.
- ☐ Disordered eating screen completed per WL-003.
- ☐ Baseline laboratory panel drawn and reviewed per WL-009.
- ☐ Informed consent covering off-label or compounded status where applicable.

PREGNANCY PLANNING

Semaglutide is not used in pregnancy and should be discontinued well in advance of a planned conception because of its long half-life — your medical director must specify the discontinuation interval used by the practice. Note also that weight loss and delayed gastric emptying can affect oral contraceptive reliability; counsel patients accordingly and document it.

3 Titration Schedule

The escalation schedule below follows the labeled approach for weight management. **Verify against current manufacturer labeling before adoption**, and have your medical director confirm the schedule used for any compounded preparation, which may differ in concentration and presentation.

STANDARD WEIGHT-MANAGEMENT ESCALATION

Phase	Weekly dose	Minimum duration	Notes
Initiation	0.25 mg	4 weeks	Tolerance-building dose. Not intended to produce meaningful weight loss.
Escalation 1	0.5 mg	4 weeks	Assess tolerance before advancing.
Escalation 2	1.0 mg	4 weeks	Many patients respond adequately here.
Escalation 3	1.7 mg	4 weeks	Advance only if response is inadequate and tolerance is good.
Maintenance	2.4 mg	Ongoing	Maximum labeled weight-management dose. Do not exceed.

FOUR WEEKS IS A FLOOR, NOT A TARGET

The minimum interval exists to build tolerance and reduce GI adverse effects. Escalating faster because a patient is impatient or a plateau feels slow is the most common avoidable error in this category. If a patient requests faster escalation, decline, explain, and document both. See WL-004.

4 Administration & Storage

- 1

Confirm the product and concentration against the prescription before drawing or dispensing. Concentration varies between branded pens and compounded vials, and this is where the serious dosing errors happen.
- 2

Verify the dose in milligrams, then convert to volume. Never accept a dose expressed only in syringe 'units' — insulin-syringe unit markings are a volume scale, not a semaglutide dose, and mixing the two has caused overdoses.
- 3

Select the injection site — abdomen, thigh or upper arm — and rotate sites between doses. Record the site used.
- 4

Inject subcutaneously using appropriate technique. Same day each week; the time of day does not matter.
- 5

Record product, manufacturer or compounding pharmacy, lot number, beyond-use or expiry date, dose, site and administering clinician.
- 6

Train the patient if self-administering: technique demonstrated, teach-back obtained, written instructions supplied, sharps disposal explained. Record the training.

Item	Standard
Storage	Refrigerated per labeling. Do not freeze; discard if frozen. Protect from light.
In-use dating	Per manufacturer labeling for branded product; per the compounding pharmacy's beyond-use date for compounded preparations. Whichever is shorter governs.
Cold chain	Temperature log maintained for the practice refrigerator, checked and recorded daily.
Excursion	Any out-of-range event assessed and documented; product quarantined pending a disposition decision by the medical director.

Missed doses

- A dose missed by a short interval may generally be taken as soon as remembered, then the usual weekly schedule resumed — confirm the specific window against current labeling.
- If more than two consecutive weekly doses are missed, do not simply resume the previous dose. Re-initiation at a lower dose and re-titration is usually required because tolerance is lost.
- Record every missed dose, the gap length, and the restart decision with its reasoning.

5 Ongoing Assessment

At every visit	Assess and record
Response	Weight, percentage change from baseline, and progress against the goal agreed at intake.
Tolerance	GI symptoms graded per WL-005. Ask specifically rather than waiting for the patient to volunteer.
Vitals	Blood pressure and resting heart rate. A sustained rise in heart rate is a known effect and warrants attention.
Nutrition	Protein intake and adequacy of oral intake. Very low intake is a clinical finding, not a success.
Red flags	Severe abdominal pain radiating to the back, persistent vomiting, signs of gallbladder disease, vision change in diabetic patients.
Adherence	Doses taken, missed doses, self-administration technique if applicable.

STOP AND REFER TODAY

Severe persistent abdominal pain, particularly radiating to the back and with vomiting, requires same-day medical evaluation to exclude pancreatitis. Hold the drug. Do not reschedule the patient for next week.

ANESTHESIA AND SEDATION

Delayed gastric emptying raises aspiration risk under sedation or general anesthesia. Instruct every patient to inform any surgeon, anesthesiologist, dentist or endoscopist that they are on a GLP-1 agent, and record that you gave that instruction. This applies to procedures in your own practice too.

6 Documentation Requirements

- Candidacy criteria met, with the specific findings that support the decision.
- Contraindication screen: the questions asked and the answers given.
- Baseline labs, vitals and measurements with dates.
- Informed consent, including off-label or compounded disclosure where applicable.
- Every dose: date, product, source, lot, beyond-use date, amount, site, administrator.
- Every escalation decision with the criteria applied — and every decision not to escalate.
- Adverse effects reported, grading applied, and action taken.
- Patient education delivered, including the anesthesia warning and sharps disposal.
- Follow-up interval set, and action taken if the patient does not attend.

7 Quick Reference

QUICK REFERENCE CARD

Semaglutide — every visit

1. **Weight, BP, resting heart rate.** Measured, not reported.
2. **Ask about GI symptoms directly.** Grade per WL-005.
3. **Confirm protein and fluid intake.** Very low intake is a finding.
4. **Escalate only at 4+ weeks** and only against the criteria in WL-004.
5. **Record product, source, lot and BUD** for every dose.
6. **Severe abdominal pain radiating to back → hold and refer same day.**
7. **Remind: tell any anesthetist or surgeon** you are on a GLP-1.

Print, laminate and post at the point of care

8 Medical Director Authorization

This protocol is adopted as a standing order for the practice named below. By signing, the medical director confirms the protocol has been reviewed against current clinical guidance, applicable state scope-of-practice rules, and the training level of the staff authorized to perform it.

PRACTICE NAME	LOCATION / SITE
MEDICAL DIRECTOR — PRINTED NAME & CREDENTIALS	LICENSE NUMBER & STATE
SIGNATURE	DATE ADOPTED / NEXT REVIEW DATE

Template notice. This document is an editable template provided for educational and operational guidance. It is not legal advice, does not constitute a physician–patient relationship, and does not guarantee regulatory compliance or the outcome of any inspection. Drug doses, concentrations and device settings must be verified against current manufacturer labeling and prevailing clinical guidance before use. A licensed medical director must review, customize and sign this protocol before it is placed into service, and must confirm that every task assigned within it falls inside the scope of practice of the personnel performing it in your state.

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